Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

Op. Dr. Murat Ayata Private Clinic
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INFORMED CONSENT FORM

Upper / Lower Blepharoplasty

Eyelid surgery

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Planned area: ☐ Upper eyelids ☐ Lower eyelids ☐ Upper + lower eyelids

Lower lid technique: ☐ Transconjunctival ☐ Subciliary (below the lashes) ☐ Not applicable _____

Additional procedures: ☐ Fat repositioning ☐ Canthopexy / canthoplasty (lid support) ☐ None _____

Indication: ☐ Cosmetic ☐ Functional (narrowed field of vision) ☐ Both _____

Patient's concerns and expectations: _____

1 Your condition and the aim of the procedure

With age, eyelid skin loses elasticity, the muscle around the eye relaxes and the fat in the eye socket may bulge forward. As a result, **excess, drooping skin** develops on the upper lids, and **bags and hollows** develop under the eyes. Genetic tendency, sun exposure, smoking and weight changes can speed up this process.

Blepharoplasty aims to give the eye area a **fresher, more rested and natural** appearance by removing excess skin, muscle and fat, or by repositioning fat. Upper lid hooding can sometimes be severe enough to narrow the upper part of the field of vision; in that case the operation also has a functional purpose of **widening the field of vision**.

Blepharoplasty on its own does not correct a low brow position, a droopy lid caused by weakness of the lid-lifting muscle (ptosis), fine wrinkles (crow's feet) or dark circles under the eyes; these may require different treatments.

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2 How is the procedure performed?

Before surgery your eyes are examined — vision, tear production and lid function are assessed — and photographs are taken. If necessary, an assessment by an ophthalmologist will be requested.

1. **Marking:** The amount of skin to be removed is carefully marked with you sitting upright, with your eyes open and closed.
2. **Upper lid:** The incision is placed in the natural lid crease so that it lies hidden in the fold when your eyes are open. Excess skin, and if needed a thin strip of muscle and bulging fat, is removed.
3. **Lower lid – transconjunctival technique:** The incision is made on the inner surface of the eyelid, so no scar is visible outside. The fat causing the bags is removed or repositioned to fill the hollow under the eye.
4. **Lower lid – subciliary technique:** If there is also excess skin, the incision is made just below the lashes and excess skin and muscle are removed. The scar usually becomes inconspicuous over time.
5. **Lid support (canthopexy/canthoplasty):** If the lower lid is lax, supporting stitches are placed at the outer corner of the eye to prevent the lid being pulled down after surgery.
6. **Closure:** The incisions are closed with very fine stitches, which are usually removed on **day 5–7**. Protective ointment is applied to the eye.

ANAESTHESIA	APPROXIMATE DURATION	HOSPITAL STAY / DISCHARGE
Local anaesthesia (± sedation) or general anaesthesia	About 1–3 hours	Usually same-day discharge; one night may be needed after general anaesthesia

3 Expected benefits and likelihood of success

- A fresher, more rested and younger-looking eye area
- Less heaviness of the upper lids; easier vision if hooding narrows the field of vision
- Reduced lower lid bags and a smoother transition under the eye

Most patients are satisfied with the result, and the effect usually lasts many years; however, as ageing continues, the skin may relax again over time. About **2–5 in 100** patients may need a small touch-up procedure because of asymmetry, under-correction or scarring. The final result usually appears within **3–6 months**.

4 Other treatment options

- **Not having surgery:** As this is a cosmetic procedure, you always have the option not to have it.
- **Tear trough filler:** May temporarily camouflage a mild hollow under the eye; it does not correct bags or excess skin and has its own specific risks.
- **Brow lift with botulinum toxin:** In mild cases, lifting the brow by a few millimetres may open the upper lid slightly; the effect is temporary.
- **Brow lift (temporal lift / forehead lift):** If the underlying problem is a low brow, this may be more suitable instead of, or in addition to, blepharoplasty.
- **Skin resurfacing treatments (laser, chemical peel, radiofrequency):** May improve fine wrinkles and skin quality; they do not remove significant excess skin or bags.

5 What happens if the procedure is not performed?

If surgery is not performed, the current appearance of your eyelids will remain and may slowly progress with age. If upper lid hooding narrows your field of vision, this may increase over time; however, it usually poses no urgent danger to eye health.

You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time

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before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

6 Preparing for the procedure

- Blood tests are carried out before surgery; if general anaesthesia or sedation is planned, you will be assessed by the anaesthetist and will need to fast beforehand.
- Be sure to tell us if you have **dry eyes, glaucoma, thyroid disease or have had previous eye surgery (e.g. laser vision correction)**.
- If you wear contact lenses, do not wear them on the day of surgery; you will need to wear glasses after surgery.
- Do not wear eye make-up, eyelash extensions or face cream on the day of surgery. After sedation or general anaesthesia you must not drive; please go home with a companion.
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.
- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

7 Recovery: what to expect

First 48 hours	There may be swelling, tightness, slight oozing and bruising around the eyes. Pain is usually mild. Sleeping with your head raised and applying **cold compresses** help to reduce swelling. Watery eyes, sensitivity to light and temporary blurred vision (due to the ointment) may occur.
Week 1	Stitches are usually removed on **day 5–7**. Bruising and swelling decrease markedly. Eye drops/ointment are used as advised. Most patients return to desk work and social life within 7–10 days; bruising can be covered with make-up.
Weeks 2–4	Remaining swelling decreases. Contact lenses can usually be worn after **2 weeks**. Strenuous exercise and work involving bending should be postponed for about 2–3 weeks. Wait for your doctor's approval before swimming or wearing eye make-up.
Months 1–6	The redness of the scar gradually fades and the scar becomes hard to see in the lid crease. The final shape and symmetry of the lids usually settle within **3–6 months**.

8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

Expected, temporary effects (not complications — a normal part of healing)

- Swelling and tightness around the eyes; bruising, **if it occurs**, settles within 1–2 weeks
- Watery eyes, a dry or gritty feeling, stinging and sensitivity to light (first few days)
- Mild difficulty closing the eyes completely in the first few days

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- Redness, slight firmness and numbness along the incision

Common conditions (most are temporary or resolve with treatment)

- **Dry eye** — About **10–25 in 100** patients have a stinging, burning or dry feeling; it usually improves with eye drops within a few weeks to a few months. It may be more marked and longer-lasting in people who already have dry eyes.
- **Swelling of the white of the eye (chemosis)** — Swelling of the membrane covering the white of the eye with fluid, seen particularly after lower lid surgery. It usually resolves with drops and compresses within a few weeks.
- **Inability to close the eyes fully (lagophthalmos)** — Incomplete closure of the eyes during sleep is common in the first few weeks after upper lid surgery and usually resolves on its own. A night-time ointment is recommended to protect the eye during this period. It is rarely permanent (**fewer than 1 in 100**).
- **Small cysts (milia) and stitch marks** — Small white spots may form along the incision; they resolve on their own or are removed with a simple procedure.

Uncommon conditions

- **Lower lid pulled down or turned outwards (lid retraction, ectropion)** — After lower lid surgery, in about **1–5 in 100** patients (more often with the subciliary technique), the lower lid may be pulled down, more of the white of the eye may show below the iris, or the lid may turn outwards. Most cases improve with massage and taping; if it persists it may cause dry and watery eyes and **corrective surgery** may be needed.
- **Asymmetry and under- or over-correction** — Differences in crease height or shape may remain between the two eyelids; if too much skin is removed, closing the eyes may be difficult, and if too much fat is removed, a hollow appearance may develop. A correction procedure may be needed.
- **Change in the shape of the eye corner** — Where lid support (canthopexy/canthoplasty) is performed, the outer corner of the eye may look tight, tilted upwards or rounded for a while, or may differ between the two sides. In most cases this softens over a few weeks to months as the swelling settles; occasionally a correction is needed.
- **A droopy lid (ptosis) becoming apparent** — Weakness of the lid-lifting muscle that was previously hidden by excess skin may become apparent after surgery; it may need to be corrected with a separate operation.
- **Noticeable or thickened scar** — Eyelid skin usually heals very well; rarely the scar may remain visible, or a fine web may form at the inner corner (webbing).
- **Bleeding and blood collection (haematoma)** — A collection of blood under the skin occurs in about **1 in 100** patients; it usually resolves on its own, but if large it may need to be drained.
- **Infection** — Because the eyelids have a very good blood supply, infection is rare (**fewer than 1 in 100**); if it develops it is treated with antibiotics.

Rare or very rare, but potentially serious conditions

- **Bleeding behind the eye and loss of vision** — This is the most serious but very rare complication. Bleeding behind the eye occurs in about **1 in 2,000** patients and can press on the optic nerve. If it is not recognised quickly and treated as an emergency (opening the stitches, draining the blood, medication, emergency surgery), it can cause **permanent loss of vision**. Permanent loss of vision has been reported in about **1 in 10,000–40,000** patients or fewer. This is why the warning signs in the first 48 hours are so important.
- **Double vision** — Caused by a small muscle that moves the eye (inferior oblique muscle) being affected during lower lid surgery; it is usually temporary, very rarely permanent, and may require further treatment or surgery.
- **Corneal scratch or ulcer** — A scratch may occur on the clear front surface of the eye, causing pain, a gritty feeling and watering; it usually heals with drops within a few days. Very rarely long-term eye treatment may be needed.
- **Tear drainage problems** — Watering due to involvement of the tear duct is very rare.

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- **Psychological dissatisfaction** — Despite a technically successful result, a person may find it hard to adjust to their new appearance. Discussing expectations openly before surgery reduces this risk.

General risks that apply to any surgical / medical procedure

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.
- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.
- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

9 When to contact us immediately

- **Severe, increasing pain in the eye**, especially on one side
- **The eye bulging forwards**, or rapidly increasing, tense swelling and bruising of the eyelids
- **Reduced vision**, darkening of vision, double vision or restricted eye movement
- Eye pain together with nausea or vomiting
- **Fever above 38.5 °C**, increasing redness, discharge or pain at the incision
- Marked sensitivity to light and a gritty feeling that does not go away
- **If any of these occur in the first 48 hours, minutes matter: call immediately or go to the nearest emergency department.**

If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).

10 Medication and everyday-life advice

- Apply cold compresses at regular intervals for the first 48 hours; sleep on your back with your head raised.
- Do not rub your eyes. In the first few days avoid heavy lifting, bending, straining and activities that increase the risk of bleeding.
- Use the eye drops and ointments you are given as often as recommended; if your eyes do not close fully, always apply the night-time ointment.

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- As a rule, do not wear contact lenses for 2 weeks or eye make-up for 1–2 weeks. Wear sunglasses outdoors and take frequent breaks from screens.
- If you have travelled from abroad, flying is usually safe after the stitches are removed (day 5–7).
- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.
- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.
- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

12 Photographs, image records and your personal data

Photographs/videos may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

☐ I agree that my images are used **only for medical records and treatment follow-up**.

☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).

☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.

☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

13 Information and notes specific to you (completed by the doctor)

Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write “none”.

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14 Patient declaration

- ☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.
- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.
- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.
- ☐ I understand that blepharoplasty on its own does not correct a low brow position, a droopy lid caused by weakness of the lid-lifting muscle, fine wrinkles or dark discolouration under the eyes.
- ☐ I understand that if I develop severe eye pain, swelling or reduced vision in the first 48 hours after surgery, I must seek help **immediately**.
- ☐ I understand that this procedure is **cosmetic**, that it is not medically necessary and that I always have the option not to have it. I have discussed my expectations with my doctor, and the realistic result that can be achieved has been explained to me.
- ☐ I understand that perfect symmetry or an exact copy of the appearance in a photograph cannot be guaranteed, that the final result depends on how my tissues heal, and that in some cases a correction procedure may be needed.

- ☐ **I GIVE MY CONSENT to the procedure described above.**
- ☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."

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15 Signatures

	FULL NAME	SIGNATURE	DATE / TIME
Patient			
Legal representative (parent / guardian) — relationship:			
Doctor providing information and performing the procedure			
Interpreter (if needed) — language:			
Witness (if needed)			

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.

☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.